

DIGITALAIKART

PREMIUM DIGITAL GUIDES FOR INDIA

The Health Insurance Claim Playbook

Indian Family Edition

File, fight and win health insurance claims under the new IRDAI rules — every deadline, every document, every escalation route, ready to use.

19

pages of step-by-step
claim strategy

20+

IRDAI deadlines & rules
you can enforce

12

rejection reasons decoded
with your counter

6

ready-to-use templates
+ emergency card

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Verified against IRDAI regulations, the 29 May 2024 Master Circular on Health Insurance Business, and the official IRDAI / Council for Insurance Ombudsmen guidance, current to 27 September 2026.

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Read this before your family needs it

A hospital bill of ₹2 lakh arrives with a warning attached: this is the moment your health insurance policy will be tested. Roughly 8% of health insurance claims in India are repudiated — about one in twelve — and the single biggest reason families lose money is not that the treatment was uncovered, but that nobody in the room knew the rules the insurer must follow. This playbook puts those rules in your hands, in plain English, with fill-in templates you can use the same day.

Everything here is built on India's current regulatory framework: the IRDAI (Insurance Products) Regulations 2024, the IRDAI Master Circular on Health Insurance Business dated 29 May 2024, the "Cashless Everywhere" circular of 31 January 2024, the Insurance Ombudsman framework, and the Consumer Protection Act 2019. Every timeline, deadline and escalation route in this book is real, current, and enforceable. Sources are listed on the final page.

How to use this playbook

- If you have a policy — do the 30-minute "Day One" audit in Part 2 today, and fill the Family Emergency Claim Card in Part 6. Both prevent most disputes before they start.
- If a claim is happening now — go straight to Part 3 (filing it right) and the first-60-minutes card on its last page.
- If a claim is already rejected or stuck — Part 4's rejection counters and Part 5's escalation ladder, with templates in Part 6, are built exactly for your situation.
- If you are buying a policy or switching insurers — Part 7 shows how to keep the waiting periods, no-claim bonus and moratorium protection you have already earned.
- Keep one copy on every family phone. The deadlines table in Part 8 is the page you will photograph at a hospital billing counter.

PART 1

Why claims fail — and why that is fixable

Health insurance is now India's fastest-growing source of insurance complaints, and the problem is getting worse, not better:

~8% of health insurance claims were repudiated in 2024-25 — roughly 1 in 12 filed	73,729 health-claim complaints on the Bima Bharosa portal in FY 2025-26 (till Feb 2026), up from 64,365 in FY 2024-25	95% of health complaints at the Insurance Ombudsman concern claim rejections
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The Finance Ministry told Parliament in March 2026 that complaints on IRDAI's Bima Bharosa portal rose from 47,658 in FY 2023-24 to 64,365 in FY 2024-25, and had already reached 73,729 by February 2026. In FY 2024-25, grievances in general and health insurance jumped 41% in a single year to 1.37 lakh. The Council for Insurance Ombudsmen's annual report found that 95% of the health complaints it receives deal with claim rejections — total repudiation, or partial deductions from the claimed amount.

Here is the encouraging part. When policyholders escalate with documents in order, a large share of these "rejections" are reversed — through mediation in roughly 42% of cases at the most active Ombudsman centres, and by binding award in many more. The Mumbai Insurance

Ombudsman has said publicly that in most health insurance complaints, “ignorance is the root cause rather than mala fide intent”. The system is not rigged against you; it simply rewards the people who know the rules.

WHAT CHANGED IN 2024 — AND WHY IT IS YOUR UNFAIR ADVANTAGE

The IRDAI Master Circular on Health Insurance Business dated 29 May 2024 consolidated over 55 older circulars into one policyholder-first rulebook. It cut the moratorium period (after which your policy cannot be questioned except for proven fraud) from 8 years to 5, capped the pre-existing disease wait at 36 months, forced 1-hour cashless decisions and 3-hour discharge approvals, and banned claim rejections without a senior committee’s approval. Most families have never read these rules. Insurers know that. After this book, you will be the exception.

Your unfair advantages, once you know the rules

- The 1-hour / 3-hour rule: a cashless request must be decided within 1 hour, and discharge authorisation given within 3 hours — or the insurer pays the hospital’s delay charges, not you.
- The committee rule: no health claim can be repudiated without the approval of the insurer’s Claims Review Committee (a sub-group of its Product Management Committee), and the rejection letter must cite the exact policy clause. A TPA desk or junior officer cannot legally reject your claim on their own.
- The 30-day / 45-day rule: a reimbursement claim must be settled or rejected within 30 days of the last necessary document — extendable to 45 days only where an investigation is genuinely warranted, after which the insurer owes you interest at 2% above the prevailing bank rate.
- The 60-month moratorium: after five years of continuous coverage, your policy can no longer be questioned for non-disclosure or misrepresentation except in proven fraud. This single rule defeats the most common rejection of all.

PART 2

Know your policy before you need it

Almost every nasty claim surprise was visible in the policy document from day one. Sit down with your policy PDF — or ask your insurer’s app or helpline for it — and complete this audit once. It takes about 30 minutes per family member covered.

The seven things to pull out of your policy

1. The Customer Information Sheet (CIS). Every policy under the 2024 framework must come with a one-page CIS in simple language: what is covered, exclusions, sub-limits, deductibles, co-payments, waiting periods and the claims procedure. If you never read the policy, at least read this page — the Master Circular requires insurers to provide it with every policy document.
2. Waiting periods. The maximums are now capped by regulation: an initial waiting period of 30 days from the start of the policy; pre-existing diseases capped at 36 months; specified diseases and procedures (like joint replacement or cataract) capped between 24 and 36 months depending on the product; maternity as per your policy’s own terms. Write down, for each family member, the exact date each waiting period ends.

3. Pre-existing disease (PED) definition. A PED is any condition diagnosed, or for which medical advice or treatment was received, within 48 months before your policy started. The 36-month PED waiting period applies to those conditions. When in doubt, disclose — see the non-disclosure trap below.
4. Sub-limits, co-payments and deductibles. A room-rent sub-limit (for example, 1% of sum insured per day) silently shrinks the entire claim, because hospitals often bill every charge in proportion to your room category. Note every sub-limit and co-pay percentage now, not at the billing counter.
5. Exclusions. Standard exclusions are now uniform across insurers, but product-specific ones differ. List what your policy permanently excludes (e.g. cosmetic surgery without medical necessity) and what it covers after a waiting period.
6. Network hospitals and the “Cashless Everywhere” right. Save your insurer’s network hospital list, but also know this: since the 31 January 2024 circular, you can claim cashless treatment even at a non-network hospital, provided it is registered under the Clinical Establishments Act or applicable state law — ideally with intimation about 48 hours before a planned admission.
7. Contacts. Save in every family member’s phone: the insurer’s 24x7 helpline, the TPA name and number, the Grievance Redressal Officer’s email (it must be in the policy and on the insurer’s website), and the Bima Bharosa portal address. Then fill the Family Emergency Claim Card in Part 6.

THE NON-DISCLOSURE TRAP — THE #1 REASON CLAIMS DIE

Insurers reject claims on “non-disclosure of material facts” — a BP reading from 3 years ago, a paediatric consultation, mild diabetes not mentioned at proposal. Two protections: (1) Disclose everything you reasonably know when buying or renewing; a slightly higher premium or a PED waiting period is far cheaper than a rejected ₹5 lakh claim. (2) After 60 months of continuous coverage, the moratorium rule protects you — the policy cannot be contested for non-disclosure or misrepresentation except in proven fraud. A lapsed renewal breaks this clock, so never let it lapse.

The waiting-period map (maximums under the 2024 regulations)

Waiting period	What it blocks	Maximum cap
Initial waiting period	Any hospitalisation in the first days of a new policy (accidents are usually exempt)	30 days
Pre-existing diseases	Treatment linked to conditions diagnosed/advised in the 48 months before the policy	36 months
Specified diseases & procedures	Named diseases/procedures such as cataract, joint replacement, hernia	24–36 months
Maternity & newborn	Delivery and maternity-related expenses, as per your policy’s terms	As stated in policy
Moratorium (not a wait)	After this, no contesting the policy except proven fraud — includes ported time	60 months continuous

A waiting period you have already served carries over when you renew, migrate or port your policy — see Part 7. That is why continuity matters more than almost any other single decision you make with health cover.

No-claim bonus: how claim-free years grow your cover

For every claim-free year, insurers add a cumulative bonus to your sum insured — commonly 5–10% per year, capped (often around 50–100% of the base sum insured, depending on the product). Two practical consequences families miss:

- One small claim can cost more than it pays: it resets or reduces the accumulated bonus and the restored amount. Before filing a ₹6,000 reimbursement claim in year five, check the bonus at stake — for very small claims, self-funding is often cheaper. (Insurers increasingly offer bonus-protection riders that allow one small claim a year; check yours.)
- The bonus is part of your accrued credits: it carries forward on renewal, migration and portability, and counts when the moratorium clock and waiting periods are assessed against the enhanced cover. If the sum insured is increased mid-journey, the 60-month moratorium applies separately to the enhanced portion.

Pre- and post-hospitalisation: the claim windows families forget

Most policies pay expenses for a period before admission (commonly 30 days) and after discharge (commonly 60 days) — diagnostics, consultations and medicines connected to the hospitalisation. They are claimed with the main claim, on prescriptions and bills from those windows, so start a folder the day someone falls ill, not the day of admission. Check your policy's exact windows and claim them every time: this is money families routinely leave on the table.

The Day One Audit Worksheet — print and fill today

One sheet per policy. Once this is filled, the Emergency Card in Part 6 takes ten minutes instead of an hour.

Question	Your answer
Policy number / insurer / TPA	
Sum insured (+ bonus, if any)	
Initial 30-day wait ends on	
PED wait (max 36 months) ends on	
Specified-disease waits end on (list)	
Moratorium (60 continuous months) reached on	
Room-rent sub-limit / co-pay / deductibles	
Pre- and post-hospitalisation windows	
Nearest network hospitals (2)	
Insurer 24×7 helpline	
Grievance Redressal Officer email	
Next renewal date (set reminder 15 days before)	

Ten myths that cost families money

“The hospital desk said no, so the claim is rejected.”

A cashless denial is not a claim repudiation — pay and file reimbursement. Only the insurer’s written repudiation letter, with CRC approval and a cited clause, is a rejection.

“They can reject for anything I forgot to mention.”

After 60 continuous months, non-disclosure and misrepresentation cannot be used against you except in proven fraud.

“TPA rejected my claim.”

A TPA administers claims; it cannot repudiate them. Only the insurer’s Claims Review Committee can, and only with a written, clause-cited letter.

“Old policies never pay.”

Public-sector insurers’ claims settlement by amount has historically run ahead of most private players; what decides outcomes far more is documentation and escalation.

“It is too small an amount to fight.”

The entire escalation ladder — GRO, Bima Bharosa, Ombudsman — is free. Partial deductions of a few thousand rupees are among the most commonly reversed complaints.

“My agent will handle it.”

Agents sell policies; they have no standing in your grievance. The complaint must come from you, the insured or claimant.

“I missed renewal by a week, so I lost everything.”

You have a grace period — 15 days (monthly premium mode) or 30 days (quarterly/half-yearly/annual) — and renewal within grace protects your accrued credits.

“Cashless is only at network hospitals.”

Since the Cashless Everywhere circular of 31 January 2024, cashless is available even at many non-network hospitals registered under the Clinical Establishments Act or state law.

“Switching insurers means starting from zero.”

Portability carries your served waiting periods, no-claim bonus and moratorium time to the new insurer. The old insurer must transfer your data within 72 hours; the new one must decide within 5 days.

“Complaining does nothing.”

Bima Bharosa tokens are tracked to resolution, and an Ombudsman award that the insurer ignores attracts a penalty of ₹5,000 per day. Ignoring an informed policyholder is now expensive for insurers.

PART 3**Filing a claim the right way**

Claims are won and lost on process, not luck. Follow these sequences in order and keep the paper trail.

Cashless at a network hospital — planned admission

- T-minus 48 hours: ask the hospital's insurance desk to initiate pre-authorisation at least 48 hours before admission. For a non-network hospital, intimation roughly 48 hours in advance activates your Cashless Everywhere right.
- What gets sent: the pre-authorisation form, doctor's advice for admission, estimated cost, and your policy and ID details. Ask the desk for a stamped copy of everything sent.
- The 1-hour rule: the insurer must communicate its pre-authorisation decision within one hour of receiving the request. If it is delayed beyond that and the hospital levies extra charges for the delay, the Master Circular puts those charges on the insurer, not you.
- Partial approval? Pre-authorisation covers an estimated amount. Mid-treatment, the hospital can send an enhancement request with fresh medical justification — insist that it is sent, in writing.
- At discharge: the insurer must give final authorisation within 3 hours of the hospital's discharge request. Before leaving, collect every original bill, the discharge summary, prescriptions, and investigation reports — originals are the currency of any later dispute.

Emergency admission

- Pre-authorisation is still required for cashless, but in an emergency the hospital desk initiates it after stabilisation; the one-hour decision clock applies. Get the claim intimated to the insurer or TPA at the earliest — a phone intimation followed by the written claim is fine.
- If the hospital is not in the network, treatment is still claimable: pay and file for reimbursement, or invoke Cashless Everywhere where the hospital is registered under the Clinical Establishments Act or state law.

CASHLESS DENIED IS NOT CLAIM REJECTED

A cashless refusal is only a refusal to pre-authorise — it is not a repudiation of your claim. The correct sequence: get the denial note and its stated reason in writing, pay the hospital, collect the discharge summary and itemised bills, and file the same claim as a reimbursement claim. Many claims denied at the cashless stage on thin grounds — “no active line of treatment”, “OPD sufficient” — are paid on reimbursement once the treating doctor's justification is on record. The denial note itself becomes evidence if you escalate.

Reimbursement claim — step by step

1. Intimate early. Notify the insurer/TPA within the window stated in your policy (commonly 24–48 hours for emergency admission; before discharge otherwise). Note the date, time and reference number.
2. Collect the full document set (checklist below). Originals matter — keep your own scanned copies of everything.
3. Submit with a written list. Hand over or email the set with a covering list of every document, and get an acknowledgement with a date. This date starts the insurer's 30-day clock.
4. Respond once, completely. If the insurer asks for documents, send everything together. Except where fraud is suspected, the insurer should not demand documents endlessly in pieces — documents not listed in the policy terms are not “necessary” documents.
5. Hold the deadline. Settlement or rejection must come within 30 days of the last necessary document; if they investigate, the investigation must finish within 30 days and the claim be

settled within 45. Beyond that, the insurer owes interest at 2% above the prevailing bank rate — put it in writing (Template 2).

The document checklist

Tick every box before you submit a reimbursement claim (and keep scanned copies of all of them):

- ☐ Filled, signed claim form
- ☐ Policy copy (or policy number on every page of the submission)
- ☐ Discharge summary from the hospital — original, signed and stamped
- ☐ Final hospital bill — original, itemised, with payment receipt
- ☐ All indoor case papers: daily treatment records, nurse's notes, OT notes if surgery
- ☐ Prescriptions and diagnostic/investigation reports during hospitalisation
- ☐ Pharmacy bills and implant invoices with prescriptions/notes
- ☐ Pre- and post-hospitalisation prescriptions, bills and reports (check your policy's covered window)
- ☐ KYC: photo ID, cancelled cheque or bank passbook for the payout
- ☐ For accident cases: FIR or medico-legal certificate, where applicable

THE CLAIM DIARY — TEN MINUTES THAT WIN DISPUTES

From the first phone call, keep one page per claim with: date, time, who you spoke to (name and designation), what was said, and the reference number of the call. A written contemporaneous record is treated as evidence in grievances and by the Ombudsman; "they told me on the phone" is not.

PART 4

When things go wrong

Every repudiation letter must, by rule, state the specific reason and point to the exact policy clause it relies on. Match the reason to your counter below — then send Template 1.

Non-disclosure of pre-existing disease

Your counters: (a) was the condition actually diagnosed or treated within the 48 months before the policy — if not, it is not a PED; (b) did you disclose it in the proposal — produce the proposal form; (c) if the policy has run 60 continuous months, the moratorium bars this ground except proven fraud. Ask in writing for the evidence they relied on and the Claims Review Committee's approval date.

"Hospitalisation not necessary — OPD was enough"

Demand the insurer's own medical opinion against the treating doctor's. Policy terms define admissibility; they do not override clinical judgment. Get the treating consultant's written justification for admission (severity, injections needing monitoring, inability to manage at home) and attach it to the appeal. This ground softens frequently at the Ombudsman.

“No active line of treatment”

Most common in cashless denials. This is a pre-authorisation view, not a final repudiation — pay, then file reimbursement with the treatment records and doctor’s notes showing active management. Keep the denial note as evidence.

Waiting period not served

Check the exact dates: the initial 30-day wait, the 36-month PED cap, the 24–36 month specified-disease waits. If you ported or renewed continuously, waiting periods already served carry over — produce the old policy’s continuous-run evidence. For accidents, the initial waiting period generally does not apply.

Room-rent sub-limit breach

If the room charge exceeded the policy’s per-day cap, insurers proportionately cut associated charges, not the whole claim. Check whether the sub-limit was properly disclosed in the CIS. Ask for the calculation in writing; proportionate deductions are frequently mis-applied and recoverable in part.

Permanent exclusion

Compare the cited exclusion with the standardised exclusion list and your policy’s CIS. If the treatment was medically necessary and the exclusion is being read more widely than its wording, say so in the appeal. Ambiguity in a contract is read against the drafter — the insurer.

Lapsed policy

Grace period: 15 days where premium is paid monthly, 30 days for quarterly/half-yearly/annual modes. A claim within the grace period after payment, or a renewal completed in grace, preserves your accrued credits — waiting periods, no-claim bonus, moratorium. If the insurer never issued a renewal notice or premium reminder, contest the lapse in writing.

Delay in intimation or documentation

Late intimation alone is not automatic grounds for rejection — the insurer must show it was actually prejudiced by the delay. Explain the genuine reason (emergency, hospitalisation of the claimant), attach the full document trail, and cite IRDAI’s position that claims should not be repudiated for procedural delay without prejudice to the insurer’s assessment.

Investigation has no end date

There is one: investigation within 30 days of the last necessary document, settlement within 45. After that, interest at 2% above the bank rate runs. Send Template 2 and note the date.

Partial deduction without explanation

Every deduction must be explained against policy terms — the Master Circular requires reasons for partial disallowance too. Demand the item-wise breakup; unexplained deductions at the Ombudsman are routinely ordered back.

Rejection by TPA or hospital desk

A TPA administers claims; it cannot repudiate yours. Since 29 May 2024, repudiation needs the approval of the insurer’s Claims Review Committee and a letter citing the clause. Ask, in writing, whether the CRC approved your rejection — several “rejections” quietly reverse at this question.

Vague or oral rejection with no letter

If you only have a verbal refusal, write to the Grievance Redressal Officer asking for the written repudiation letter with the clause and CRC approval. The clock for every escalation right starts from the insurer's written response — make them put it on paper.

THE THREE QUESTIONS THAT START EVERY APPEAL

1. Which exact policy clause does the rejection rely on? 2. On what date did the Claims Review Committee approve this repudiation? 3. On what date did you receive my last necessary document? Send them in writing (Template 1) and give the insurer 15 days. Their answers — or their silence — become the backbone of your Bima Bharosa complaint and your Ombudsman case.

A worked example: the ₹3.2 lakh claim that came back

This composite case shows the full ladder working as designed. Names and details are illustrative; every rule and timeline in it is real and cited in this book.

The Mehta family's policy: ₹5 lakh family floater, running continuously since 2018 — past the 60-month moratorium. In March, Mr Mehta is admitted for three days; the insurer's TPA desk denies cashless with a one-line note: "no active line of treatment".

What the family did	The rule that made it work	Result
Took the cashless denial note in writing from the hospital desk before discharge	A cashless denial is not a repudiation; the note is evidence	Day 0 — paper trail begins
Paid ₹3.2 lakh by card, collected discharge summary, itemised bills, OT notes, prescriptions	Originals are required for reimbursement; scans kept	Day 1 — document set complete
Filed a reimbursement claim with a covering list; got an acknowledgement with date	The 30-day clock starts from the last necessary document	Day 3
Insurer's letter rejects the claim citing "non-disclosure of hypertension", no clause quoted	Rejections must cite the exact clause and carry Claims Review Committee approval	Day 26
Sent Template 1 (appeal) asking for the clause, the CRC approval date, and citing the 60-month moratorium	GRO must respond within 15 days	Day 28
No reply after 15 days — filed on Bima Bharosa with the letter, denial note and bills	Free portal complaint with a token number; tracked in complainant login	Day 45
Insurer offered a 60% settlement; family rejected it and filed with the Insurance Ombudsman	Ombudsman is free; within one year of the rejection	Day 60
Mediation recommended full payment minus a ₹9,000 non-payable item; insurer complied	Recommendations and awards are binding on the insurer within 30 days; ignoring them costs ₹5,000/day	Day 105 — ₹3.11 lakh paid

Total cost of the fight: zero rupees in fees. Total writing: three letters and two portal filings. The deciding factors were the written denial note, the dated acknowledgement, and knowing the moratorium rule — not luck.

How a room-rent sub-limit actually eats money

Insurers do not usually cut only the excess room charge — they apply proportionate deduction to associated medical expenses. Know the arithmetic before you pick your room:

Example. Sum insured ₹5 lakh, policy sub-limit: single private room. You take a deluxe room costing ₹8,000/day when the eligible private-room rate is ₹4,000/day. The hospital's total bill of ₹2,00,000 (room ₹48,000 + associated charges ₹1,52,000) can be reduced proportionately: since the room taken was 2× the eligible rate, associated charges are also scaled to half — the payable amount moves toward ₹48,000 + ₹76,000 = ₹1,24,000 before other deductions. The ₹76,000 hole was created by the room choice. Ask for this calculation in writing whenever it is applied — mis-application is common and recoverable in part at the Ombudsman.

HOSPITALS PLAY THIS GAME TOO

Treatment patterns often change once a patient is identified as insured — the Ombudsman and IRDAI have both flagged hospitals overbilling insured patients. Always ask for the itemised bill, question consumables and “package” upgrades before discharge, and compare the bill against the estimate you were given at admission. You are entitled to the breakup; template 5 gets you the records.

PART 5

The escalation ladder

Every rung of this ladder is free. No lawyer, no court fee. Each rung has a deadline the insurer must respect, and your right to climb appears the moment they miss it.

Step 1 — The Grievance Redressal Officer (GRO)

Every insurer must maintain a Grievance Redressal Officer at head office and branches, whose contact details must appear in your policy and on the insurer's website. Write (email is fine) with policy number, claim number, dates, and exactly what you want — the relief, in rupees. The GRO must resolve your grievance within 15 days. Take a written acknowledgement with a date.

- Attach: the repudiation letter or the delayed-claim trail, the document set, and your claim diary.
- Do not phone and consider it done. Phones are for intimation; paper is for grievances.

Step 2 — Bima Bharosa (IRDAI's grievance portal)

If 15 days pass, or the reply is a brush-off, register on bimabharosa.irdai.gov.in — the complaint flows to both the insurer and IRDAI's repository, and you get a token number to track it in your complainant login. Alternatives: email complaints@irdai.gov.in, or call the toll-free grievance numbers 155255 / 1800 4254 732.

- The portal shows the insurer's status updates and the final resolution in your login — screenshot everything.
- Only complaints from the insured or the claimant are entertained, so file it yourself, not through an agent.
- Bima Bharosa complaints are tracked to closure — that visibility alone moves stubborn files.

Step 3 — The Insurance Ombudsman

- Who: 17 Ombudsman centres across India. Your complaint goes to the centre with territorial jurisdiction over the insurer's branch or your own residential address — the list is at cioins.co.in.
- Eligibility: you have complained to the insurer in writing, and either they rejected it, you are unsatisfied with the reply, or one month has passed with no reply. File within one year of that point.
- How: a written, signed complaint (Template 4) — in person, by post or email, or online through the Council for Insurance Ombudsmen. It costs nothing. Complaints cover repudiation, delay in settlement, and partial settlements.
- What happens: if mediation suits your case, the Ombudsman's recommendation can come within a month; if not, an award is passed within three months of receiving complete requirements. The award is binding on the insurer, which must comply within 30 days — non-compliance attracts a penalty of ₹5,000 per day.
- Claim size: the route covers claims within its value limit — the Council's site currently shows ₹50 lakh (some older IRDAI pages still say ₹30 lakh; confirm on cioins.co.in before filing).

Step 4 — Consumer commissions (and courts)

If the Ombudsman's outcome does not satisfy you, or the claim exceeds the Ombudsman limit, you can approach the consumer dispute redressal commissions under the Consumer Protection Act 2019 — District (claims up to ₹50 lakh), State (₹50 lakh to ₹2 crore) and National (above ₹2 crore). A consumer complaint must normally be filed within two years of the cause of action. This route takes longer and may involve fees and, optionally, a lawyer — but the documentary record you built in Steps 1–3 is exactly what it runs on.

THE LADDER AT A GLANCE

GRO (15 days) → Bima Bharosa (token number; free) → Ombudsman (free, award in ~3 months, ₹5,000/day penalty on the insurer for ignoring it) → Consumer commission (District < ₹50L, State to ₹2Cr, National above). Never skip a rung in writing — each letter you sent is the evidence the next rung needs.

What each rung actually feels like

Knowing the law is half the fight; the other half is managing the process calmly. What to expect:

- The GRO reply: expect a templated defence first. If it restates the same clause without answering your three questions, that is your signal to climb — the unanswered questions become your Bima Bharosa complaint's opening lines.
- Bima Bharosa: the insurer now responds knowing IRDAI can see the trail. Well-documented claims often settle here — including partial settlements. Decide beforehand whether a partial

settlement is acceptable; once you accept any amount as full-and-final, reopening is hard.

- Ombudsman mediation: an informal session — often by video or at the centre — where the Ombudsman looks for a fair middle. Bring the claim diary, dated acknowledgements, and the clause-demand letters. Speak to dates and documents, not anger. If the recommendation seems fair, accept; it binds the insurer.
- Ombudsman award: if mediation fails, submit a complete written case — facts, grounds, relief sought — and the award comes within three months of your complete submission. Compliance is due in 30 days; the ₹5,000/day penalty means insurers rarely stall an award.
- Consumer commission: a formal pleading stage. Free to file, but slower and procedure-heavy; many complainants engage a lawyer here. Your Steps 1–3 record usually decides the outcome.

THE MISTAKE THAT KILLS GOOD CASES

Accepting a small “goodwill” payment with a full-and-final discharge letter, while angry and exhausted, waives the balance of the claim. Never sign a full-and-final settlement on the spot — take it home, read it against this book, and decide with a cool head.

PART 6

Ready-to-use templates

Replace everything in [square brackets]. Send by email (keeps an automatic record) and, where you can, by a trackable mode. Attach the documents you cite. Keep every reply.

TEMPLATE · 1 — APPEAL AGAINST CLAIM REJECTION (to the Grievance Redressal Officer)

When to use: Any written repudiation, within days of receiving it. One page wins most appeals.

To: The Grievance Redressal Officer, [Insurer name and office]

Subject: Appeal against repudiation of Claim No. [claim number] — Policy No. [policy number]

Respected Sir/Madam,

This is to appeal against the repudiation of my claim no. [claim number] for ₹[amount], intimated to me on [date], on the ground of [state the reason given].

In this regard, I request the following information in writing: (a) the exact policy terms and clause relied upon for the repudiation; (b) the date on which the Claims Review Committee approved this repudiation, as required by the IRDAI Master Circular on Health Insurance Business dated 29 May 2024; and (c) the date of receipt of my last necessary document.

I submit that the claim is payable for the following reasons: [set out your facts — e.g. the condition was not diagnosed or treated within 48 months before policy inception; the policy has run [N] continuous years and the 60-month moratorium applies; hospitalisation was medically necessary as per the enclosed treating consultant’s note]. Enclosed: [list documents].

Kindly re-examine and settle the claim within 15 days, failing which I will be constrained to escalate the matter on the Bima Bharosa portal and to the Insurance Ombudsman.

Yours faithfully, [Name, address, phone, email, policy number, claim number] · [Date]

TEMPLATE · 2 — REMINDER FOR A DELAYED CLAIM (interest demand)

When to use: When a submitted claim has crossed 30 days (or 45 with investigation) with no decision.

To: The Grievance Redressal Officer, [Insurer name]

Subject: Claim No. [number] pending beyond the regulatory timeline — request for settlement with interest

Respected Sir/Madam,

My reimbursement claim no. [number] for ₹[amount] was submitted on [date], and the last necessary document was received by you on [date] (acknowledgement enclosed). More than 30/45 days have since elapsed.

As per IRDAI's framework, a claim must be settled or rejected within 30 days of receipt of the last necessary document, and where investigation is warranted, within 45 days. I therefore request settlement of the claim together with interest at 2% above the prevailing bank rate for the period of delay.

If the claim is under investigation, kindly confirm the stage and the provisions under which the investigation is being conducted. An early response within 15 days will be appreciated.

Yours faithfully, [Name, phone, policy number, claim number] · [Date]

TEMPLATE · 3 — BIMA BHAROSA / GRO GRIEVANCE (short form)

When to use: When 15 days have passed since your complaint, or the reply is evasive.

Policy No.: [·] Claim No.: [·] Insurer/TPA: [·] Grievance in brief:

My claim for ₹[amount], submitted on [date], was rejected/delayed/partially deducted by ₹[amount] on the ground of [reason]. My complaint to the insurer (dated [·]) has not been resolved to my satisfaction / has received no reply for over 15 days / one month.

Relief sought: settlement of ₹[amount], with interest where the regulatory timeline has been crossed, and reasons for every deduction. Documents attached: [rejection letter, claim form, discharge summary, bills, correspondence].

Registered on bimabharosa.irdai.gov.in on [date] — Token No.: [·]

TEMPLATE · 4 — INSURANCE OMBUDSMAN COMPLAINT (format per the Ombudsman framework)

When to use: After the insurer rejects or fumbles your written complaint; within one year.

To: The Insurance Ombudsman, [Centre — see ciains.co.in for jurisdiction]

Complaint against: [Insurer name], [branch/office complained against]

Complainant: [Name, full address, phone, email] · Policy No.: [·] Claim No.: [·]

Facts: I hold policy no. [·] effective from [date]. A claim arose on [date] for ₹[amount] for [treatment]. The insurer repudiated/deducted/delayed the claim by its letter dated [·] on the ground of [reason], or has failed to reply to my representation dated [·] for over one month.

Prior representation: I made a written complaint to the insurer on [date] (copy enclosed). The insurer [rejected it on (date) / replied unsatisfactorily on (date) / has not replied for over one month].

Grounds: The repudiation is contrary to [state grounds — non-disclosure barred by the 60-month moratorium under the IRDAI Master Circular dated 29 May 2024; hospitalisation certified medically necessary; deductions not explained against policy terms; the 30/45-day timeline crossed without settlement].

Relief sought: [Settlement of ₹ amount / refund of wrongly deducted ₹ amount / settlement with interest].

Declaration: The matter is not pending before any other forum. I request the Ombudsman to take up the complaint. Place, date, signature.

Enclosures: [policy, claim forms, rejection letter, medical records, correspondence, GRO complaint]

TEMPLATE · 5 — RECORDS REQUEST TO THE HOSPITAL

When to use: Before discharge or immediately after — you need the full record set for any claim or dispute.

To: The Medical Records / Insurance Desk, [Hospital name]

Subject: Request for copies of medical records — Patient [name], UHID/IP No. [·], admission [date] to [date]

Respected Sir/Madam,

I request certified copies of the complete medical record of the above admission for insurance purposes: discharge summary, indoor case papers and daily notes, operation theatre notes (if any), consultation notes, investigation reports, and the itemised final bill with payment receipt and pharmacy/implant invoices.

Kindly provide the set within [3] working days, as they are required to file/contest an insurance claim. I am enclosing the patient's authorisation and my ID proof.

Yours faithfully, [Name, relationship to patient, phone] · [Date]

The Family Emergency Claim Card

Print this page, fill it in, and keep one photo of it on every family member's phone and one copy with your documents. In an emergency nobody remembers policy numbers.

FOR EACH POLICY	FILL IN TODAY
Insurer & policy number	
Policy type & sum insured	
24×7 helpline number	
TPA name & number	
Network hospital (nearest 2)	
Grievance officer email	
Renewal date & premium	
Members covered & ages	

FOR EACH POLICY	FILL IN TODAY
PED notes & waiting-period end dates	
Bima Bharosa	bimabharosa.irdai.gov.in · 155255 · complaints@irdai.gov.in
Ombudsman centre (my jurisdiction)	
This playbook	Templates in Part 6 · Deadlines table in Part 8

The first 60 minutes after an emergency admission

- 0–10 min: call the insurer/TPA helpline; note the reference number. Call the hospital insurance desk to the bedside.
- 10–20 min: verify policy active and claim eligible (sum insured, waiting period, network status). Hand the desk the Emergency Card numbers, not the family's panic.
- 20–40 min: ensure pre-authorisation is initiated in writing with the estimated cost; ask for the stamped request copy.
- 40–60 min: start the claim diary. Photograph the Emergency Card and the pre-auth request; WhatsApp them to yourself so they are timestamped.

PART 7

Keep the protection you have earned

A ₹10,000-per-year decision protects a ₹10 lakh claim. Three rules keep that protection alive.

1. Never break continuity

The 60-month moratorium clock, served waiting periods, and accumulated no-claim bonus all depend on unbroken continuous coverage. One lapsed year can reset protection worth lakhs. Set an auto-renewal or a calendar alarm 2 weeks before the renewal date.

2. Use the grace period correctly

If you miss the renewal date, you have 15 days of grace (monthly premium mode) or 30 days (quarterly, half-yearly, annual). Renew within grace and every accrued credit — sum insured enhancements, no-claim bonus, served waiting periods, moratorium — is protected. A claim arising within the grace window after premium payment is likewise protected.

3. Port smart — the 72-hour / 5-day rules

Unhappy with your insurer? You can port to another insurer without losing accrued benefits. Apply to the new insurer before your renewal date (ideally 45 days before). The rules that bind them: your existing insurer must transfer your full underwriting and claims history through the IIB portal within 72 hours of the request, and the acquiring insurer must decide within 5 days of receiving that information. Waiting periods, no-claim bonus and moratorium time served carry over. The same protections apply to migration between products within the same insurer.

- Porting discipline: do not let the old policy lapse while porting — apply in time, and remember the new insurer can only apply waiting periods for conditions that genuinely qualify as PED.

- Senior citizens: insurers must design products for seniors, students, maternity and other groups under the 2024 framework, and entry-age caps have been removed for standard retail products. Read the co-payment clause carefully — it is the most common cost in senior-focused policies.

WHEN THE OTHER SIDE DOES NOT PLAY FAIR

If an old policy lapsed because the insurer never sent the renewal notice they were supposed to — contest it in writing with your records. And if a TPA or desk stalls beyond every deadline in the next table, stop calling and start writing: Bima Bharosa at step one, the Ombudsman at step two. The entire ladder is free, and it works more often than families believe.

If this happens...

Your situation	Do this now	Where
Admission is being planned	Pre-authorisation 48 hours ahead; carry the Emergency Card	Part 3
Cashless denied at the hospital desk	Take the denial note in writing; pay and file reimbursement	Part 3 / Part 6
Claim rejected by letter	Template 1 to the GRO the same week; ask for clause + CRC date	Parts 4, 5
Claim pending beyond 30/45 days	Template 2 with the interest demand	Parts 4, 6
GRO silent after 15 days	File on Bima Bharosa; keep the token number	Part 5
Unsatisfied with the insurer's reply	Ombudsman complaint within one year (Template 4)	Parts 5, 6
Deductions you do not understand	Demand the item-wise breakup against policy terms	Part 4
Renewal missed	Renew inside the grace period (15/30 days) to protect credits	Part 7
Thinking of switching insurers	Port before renewal; track the 72-hour / 5-day rules	Part 7
Proposal being filled today	Disclose all medical history of the last 48 months	Part 2

PART 8

Every deadline on one page

The clock	The deadline	Who owes it
Cashless pre-authorisation decision	Within 1 hour of the hospital's request	Insurer
Final discharge authorisation	Within 3 hours of the discharge request (delay charges fall on the insurer)	Insurer

The clock	The deadline	Who owes it
Reimbursement claim decision	Within 30 days of the last necessary document	Insurer
Special investigation	Complete within 30 days; settle the claim within 45	Insurer
Delay beyond 45 days	Interest at 2% above the prevailing bank rate	Insurer
Grievance to the insurer's GRO	Resolution within 15 days	Insurer
No reply to your written complaint	Escalate to Bima Bharosa after 15 days / one month	You
Ombudsman filing window	Within 1 year of the insurer's rejection, unsatisfactory reply, or silence	You
Ombudsman award	Within 3 months of complete requirements; insurer must comply within 30 days (else ₹5,000/day penalty)	Ombudsman / Insurer
Free-look cancellation	30 days from receiving the policy	You
Premium grace period	15 days (monthly mode) / 30 days (quarterly, half-yearly, annual)	You
Portability data transfer	Existing insurer: within 72 hours via IIB; new insurer decides within 5 days	Insurers
Moratorium	After 60 months of continuous coverage — no contesting except proven fraud	Insurer
Consumer commission filing	Within 2 years of the cause of action	You

The rejection letter must state the specific reason and policy clause; repudiation requires Claims Review Committee approval. Documents beyond those listed in the policy cannot be demanded in instalments. Cashless is available even at non-network hospitals registered under the Clinical Establishments Act or state law (intimate ~48 hours before planned admission).

Glossary — the eight terms that matter

Term	What it means
Cashless	The insurer pays the network hospital directly, to the extent pre-authorised. A denial is not a claim rejection.
TPA	Third-party administrator — processes claims for the insurer. Cannot repudiate your claim.
Pre-authorisation	The insurer's approval for a cashless admission, based on an estimate. Can be enhanced mid-treatment.
Reimbursement	You pay the hospital, then claim from the insurer with documents. Decision due in 30 days.
PED	Pre-existing disease: diagnosed or advised within 48 months before the policy started. Waiting period capped at 36 months.
Sub-limit / co-pay / deductible	Caps and shares that reduce the payout. Check the CIS; ask for every deduction in writing.
Moratorium	After 60 continuous months, the policy cannot be contested for non-disclosure or misrepresentation, except proven fraud.

Term	What it means
GRO / Bima Bharosa / Ombudsman	The free escalation ladder: the insurer's Grievance Redressal Officer (15 days), IRDAI's portal, and the Insurance Ombudsman.

Sources and verification

Every rule, timeline and statistic in this playbook was verified in September 2026 against: the IRDAI Master Circular on Health Insurance Business dated 29 May 2024 (IRDAI/HLT/REG/CIR/106/05/2024) and the IRDAI (Insurance Products) Regulations 2024; the IRDAI circular on Cashless Everywhere dated 31 January 2024; IRDAI's official policyholder pages on the Insurance Ombudsman, the Bima Bharosa portal (bimabharosa.irdai.gov.in) and grievance redressal; the Insurance Ombudsman Rules 2017 framework and the Council for Insurance Ombudsmen (cioins.co.in); the Consumer Protection Act 2019; the Finance Ministry's written reply in the Lok Sabha on Bima Bharosa complaint volumes (March 2026, as reported by The Hindu BusinessLine); IRDAI Annual Report claims data as reported by The Indian Express; the Council for Insurance Ombudsmen annual report as reported by Outlook Money and The Economic Times; and current reporting by Moneycontrol, The Times of India and NDTV Profit on the 2024-26 IRDAI claim reforms.

This playbook is general information for Indian health insurance policyholders, current as of 27 September 2026. It is not legal, medical or financial advice, and it does not replace your policy's terms, which govern your specific cover. For your exact position, verify with your insurer, the IRDAI portals above, or a qualified professional.